

Hi,

The patient relapsed on Multiple Myeloma in January 2024 and was taken for consultation to PGIMER Chandigarh. They referred the patient for Bone Marrow Transplant for Multiple Myeloma to a private hospital. Henceforth the patient was taken to Amrita Hospital in early February for Bone Marrow Transplant for multiple myeloma. All this claim work has been filed for Bone Marrow Transplant to happen under Dr. Prashant Mehta in Amrita Hospital.

We are attaching all PGIMER Consultation Cards (since its a govt hospital, we only have OPD cards) and Amrita Consultation Paper from Dr Prashant Mehta. Please let us know what else is required?

Thanks,
VK Garg

BP-160/90



AMRITA INSTITUTE OF MEDICAL SCIENCES & RESEARCH CENTRE
 PAN NO: AAATM2403M, Appointment call: 01292851234
 Mata Amritanandamayi Marg, Sector 88, Faridabad, Haryana 121002

MRD No: 112145 Gender: Male DOB: 30/03/1957
 Name: VIPAN KUMAR GARG Visit Type: OP Visit Code: OP0001
 Consultant: Dr. Prashant Mehta Speciality: Medical Oncology

MEDICAL OPD RECORD

VITALS

DateTime	Temp.	Pulse Rate	Resp.	BP	GRBS	SPO2	Pain Scale	Height	Weight	BMI	BSA
10-02-2024 12:38:00	0.0	91.0	0.0	150/80	0.0	96.0	0.0	177.0	81.7	26.07	2.0

HISTORY OF PRESENTING COMPLAINTS

MM ISS II
 RISS II
 Gain 1q+
 Post
 DVCD X 16 WEEKS f/b maintenance
 Auto deferred as MRD was negative at PGIMER Chandigarh

Now Biochemical relapse
 Mild right sided chest pain, seems musculoskeletal in origin

PAST HISTORY

â?# Asthma since 20 yrs (takes foracort 200 inhaler + Montair LC regularly)
 â?# Hypertension/BP (since last 5 years) - takes medicine daily (Telmisartan (40mg)
 â?# Diagnosed as pre-diabetic in Jan 2022 (Hb1AC - 6)
 â?# Had thyroid since last 5 years but took medicines and it is under control now
 â?# Has prostate and takes Alfuzosin (10mg) daily

Multiple Myeloma History

â?# April 2022: Started complaining of back pain in April 2022 (also had covid in the same month), blood tests showed high ESR - 90
 â?# May 2022: Patient also fell one morning in the bathroom in May 2022, got unconscious (probably due to hypertension) but nothing was detected (Chest X rays done but nothing showed up)
 â?# June 2022: Bone Marrow / Serum Electrophoresis done in June 2022 and Multiple Myeloma confirmed (Patient's mother also suffered from Multiple Myeloma, Patient's mother's brother also suffered from Multiple Myeloma)
 â?# Multiple Myeloma confirmed due to Bone Lesions found in spine and some ribs (C-ve, A -ve, R-ve, B+ve in CARB diagnosis). D1 and D6 have collapsed (compression fractures) (MRI attached below)
 FISH report: Abnormal FISH results: 3-4 copies each of 1q21(CKS1B locus) and 1p32(CDKN2C locus) seen in 10% of cells suggestive of trisomy/tetrasomy of

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Name: VIPAN KUMAR GARG

Page 1 of 1

Printed On: 10/02/2024 12:38:00

Chromosome 1: Normal FISH results: No evidence of 1p32(CDKN2C) deletion, t(4;14)(p16;q32) FGFR3::IGH, t(14;16)(q32;q23) IGH::MAF, t(14;20)(q32;q12) IGH::MAFB translocations, 11q13(CCND1) rearrangement and 17p13(TP53 deletion). Note: FISH done on non-sorted sample.
â?# July 2022 - Oct 2022: Treatment Cycle of DaraVCD: Chemotherapy started on 20th June 2022 and has undergone 5 cycles. 12 Dara infusions + 16 weekly Bortezomib.
â?? Weekly dose of Bortezomib + Dexamethasone (40mg) + Cyclophosphamide + Daratumumab (400mg x 3). Patient started with Lenalidomide but was replaced with Cyclophosphamide as platelet count was going down after the patient was given Lenalidomide for 4 weeks.
â?? Weekly dose of Zoledronic acid for first 4 months
â?# Dec 2022: MRD -ve - tested NIL and no change in bone lesions in PET scan. Hence Bone Marrow Transplant was canceled since he achieved complete remission (CR).
â?# Jan 2023 - Aug 2023: Maintenance therapy of Biweekly Bortezomib 1.25mg (given subcutaneously). Quarterly Zoledronic acid.
â?# Aug 2023: Repeat PET scan and MRD test done. MRD tested negative and PET scan showed no change. Patient traveled to the US and spent 3 months with a completely normal lifestyle.
â?# Dec 2023 - Jan 2024: Myeloma follow up showed an increase in Kappa FLC to 50 that raised suspicion of relapse in Dec 23 test. Recommended to get this tested again after 1 month to see growth. Jan 2024 retested with a different lab - Kappa FLC increased again to 98.

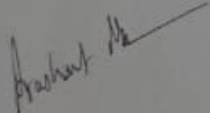
PLAN

CBC, LFT, KFT, Ca, PT/APTT
Bone marrow aspiration + Myeloma MRD - send to Dr Nupur + Bone marrow Biopsy - Oncology daycare - Dr Amit/Dr Rajesh
PET-CT Scan → V.G. (Nuclear)
DTPA scan → um caralia
2 D Echo → 3rd floor Pulmo
PFT-DLCO → 3rd floor Pulmo
Pulmonology clearance for Auto HSCT
Cardiology clearance for Auto HSCT
Medicine Consultation - for Auto HSCT
Dental Consultation
Psychiatry clearance

Follow up on 13/2/24
Pretransplant counselling on 13/2/24

Created Date: 10/02/2024

Created Time: 13:14


Signed By: Dr. Prashant Mehta
Senior Consultant
HN17785
Signed On: 10-02-2024 13:46



Embrace Good Health

NAME	VIPAN KUMAR GARG	STUDY DATE	13.02.2024
AGE / SEX	66Years / Male	MRD NO.	112145
REFERRING DEPT	OPD, Medical Oncology	Procedure	99mTc-DTPA Renal Dynamic Study
REPORTED ON	13.02.2024	REFERRED BY	Dr. Prashant Mehta

99mTc-DTPA Renal Dynamic Study

CLINICAL HISTORY: Case of multiple myeloma, post chemotherapy, plan HSCT. s.Creatinine (28.12.23): 1.26mg/dL [0.7-1.3]. Height: 177cm; Weight: 81kg; BSA: 2.01m².

PROTOCOL: Dynamic renal scintigraphy was acquired for 30minutes under the gamma camera, immediately after intravenous administration of 5.6mCi of 99mTc-DTPA. Injection Lasix (40mg) was given at the tenth minute of the study. Post void image & delayed image (at 2hours) were acquired.

FINDINGS:

Background tracer activity appears mildly elevated.

Dynamic images:

Left kidney: normal in size, shape & location with impaired perfusion & cortical tracer uptake showing slow drainage of tracer from the prominent left pelvi-calyceal system into the urinary bladder.

Right kidney: appears normal in size, shape and location with preserved perfusion & cortical tracer uptake and prompt drainage of the radiotracer from the prominent pelvi-calyceal system into the urinary bladder.

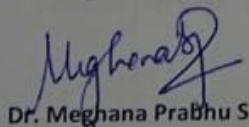
Delayed image (at 2hours): there is adequate drainage of tracer from bilateral kidneys.

Urinary bladder is visualized in the 6th minute of the study. Bilateral ureters are mildly prominent in the dynamic images.

	Left Kidney	Right Kidney
Relative Function	41%	59%
GFR : 65.7ml/min	26.8mL/min	38.9mL/min

IMPRESSION:

- **GFR: 65.7ml/min**
- **Left kidney:** Normal sized left kidney with mildly impaired cortical function and non-obstructive drainage.
- **Right kidney:** Normal sized right kidney with preserved cortical function and non-obstructive drainage.


Dr. Meghana Prabhu S

Assistant Professor

Department of Nuclear Medicine

Dr. Abhishek Behera

Assistant Professor

Department of Nuclear Medicine

(Please carry report & films on your next visit for comparison).

N.B: This is only a professional opinion and not the final diagnosis. Radiological investigations are subject to variations due to technical limitations. Hence, correlation with clinical findings and other investigations should be carried out to know true nature of illness.

MRD No: 112145

Gender: Male

Age: 66Y 10M 14D

Name: VIPAN KUMAR GARG

Visit Type: OP

Visit Code: OP0001

Service: Multiple Myeloma

RESULTREPORT

Findings

Sample ID: FCM - 67/2024

Clinical Detail: Follow up case of Multiple Myeloma

MRD Assessment time point: On maintenance Tx

Type of Sample: Bone marrow aspirate

Sample Quality: Fresh (<24 hrs) and adequate.

Methodology: Flow cytometry, Stain-Lyse-Wash (Beckman Coulter - Dx Flex)

Gating Strategy: Multicolor flow cytometry using CD45 VS side scatter gating followed by hierarchical gating.

Analysis Software: Kaluza Software (ver 2.1)

No of events acquired: 45,00,000

No of viable events (FSC Vs SSC): 96%

CD38+CD138+ Events: 0.15%

Report Summary: Flowcytometric immunophenotyping done on bone marrow aspirate shows a total of 0.15% plasma cells with immunophenotype as follows:

1. NPC (Normal Plasma Cells): 0.04%; CD45DIMCD19+CD27+CD20dimCD56-CD117- & polyclonal in distribution (κ : λ = 1.7:1)
2. APC (Aberrant Plasma Cells): 0.1%; Kappa restricted plasma cells displaying two distinct subpopulations:
subpopulation I: CD19+CD27+CD56+
subpopulation II: CD19-CD27-CD56+

Conclusion: Sample is positive for MRD (0.1%)

Comments

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MRD No: 112145

Name: VIPAN KUMAR GARG

Page 1 of 1

Reported on: 22/07/2024 11:00 AM
Reviewed by: Mr. Anil Kumar Singh

Transthoracic Echo-Doppler Report

Name	:	Mr. Vipin Kumar Garg	
Age/Sex	:	66 Years/Male	UHID : 112145
Referring Physician	:	Dr. Prashant Mehta	LAB NO : OPD
Indication	:	Multiple myeloma	Date : 10/02/2024

Measurements (mm):

	Observed Values	Normal Values
Aortic root diameter	26	20-36 (22mm/M ²)
Aortic Valve Opening		15-26
Left Atrium size	34	19-40
	End Diastole	End Systole
Left Ventricle size	40	26
Interventricular Septum	12	14
Posterior Wall Thickness	11	13
LV Ejection Fraction (%)	55%	55%-80%

Doppler velocities (cm/sec)

Pulmonary valve			Aortic valve		
Max velocity	78		Max velocity	125	
Mean PG			Max PG		
Pressure ½ time			Mean velocity		
Acceleration Time			Mean PG		
RVET			LVET		
Mitral valve			Tricuspid valve		
E	78	Max PG =	Max Velocity	45	
A	114		Mean Velocity		
DT		Mean PG =	Mean PG		
E/E'		PHT	TAPSE	1.9cm	

Mr. Vipin Kumar Garg

Regurgitation

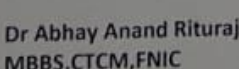
MR		TR	
Severity	Mild	Severity	Trace
Max Velocity		RVSP	26mmHg
AR		PR	
Severity	Nil	Severity	Nil
Jet width/LVOT ratio		Mean PAP	

Final Interpretation:

HR 132 BPM

1. No Regional wall motion abnormality, LVEF = 55%. (GLS = -14.9%).
2. Mild LVH present
3. Normal RV systolic function (TAPSE = 1.9cm).
4. Mild MR, Trace TR (RVSP = 26mmHg).
5. Grade I diastolic dysfunction (A>E).
6. IVC is normal > 50% respiratory variation present (CVP = 3mmHg)
7. No clot/vegetation/ pericardial pathology.


 Dr Dilip Das
 MBBS, PGDCC
 Associate Consultant
 Non Invasive
 Cardiology


 Dr Abhay Anand Rituraj
 MBBS,CTCM,FNIC
 Associate Consultant
 Non invasive Cardiology


 Dr Mohit Bhutani
 MBBS,MD,DM
 Consultant
 Cardiology


 Dr Vivek Chaturvedi
 MBBS,MD,DM
 Senior Consultant & HOD
 Cardiology



NAME	VIPAN KUMAR GARG	Embrace Good Health	12.02.2024
AGE / SEX	66Years /Male	MRD NO.	112145
REFERRING DEPT	OPD, Medical Oncology	Procedure	Whole body 18F-FDG PET-CECT
REPORTED ON	13.02.2024	REFERRED BY	Dr. Prashant Mehta

18F-FDG WHOLE BODY PET-CECT

CLINICAL HISTORY: Case of multiple myeloma ISS II, RISS II. Diagnosed in June 2022, Baseline PET/CT (13.6.22). Post 6cycles of chemotherapy, PET/CT (9.12.22): metabolic resolution of the osseous lesions. On maintenance chemotherapy till Aug 23. FDG PET/CT (at PGI, 10.8.23): no significant interval change noted. During follow-up increase in kappa FLC, suspicion for recurrence. PET/CT for further evaluation.

PROTOCOL: Whole body FDG PET CT scan was performed from vertex to feet on Siemens Biograph Horizon Time-of-Flight PET CT scanner following intravenous injection of 10mCi of 18F-FDG under standard precaution and patient preparations. Patient was asked to rest for 60min in a shielded room to allow tracer to accumulate in the body. High resolution CT scan was performed during this examination on a 16 slice MDCT with i.v injection of non-ionic contrast (low volume Visipaque, 1mL/Kg) followed by PET images. Additional brain protocol was performed. The creatinine level of the patient was 1.2mg/dl and blood sugar level was 119mg/dl at the time of tracer injection.

FINDINGS:

Overall quality of the study - Good. SUVmax mentioned in the report is in g/ml standardized to lean body mass.

BRAIN: Mild age related cortical atrophic changes are noted in bilateral cerebral and cerebellar hemispheres. The brain parenchyma is unremarkable with normal FDG biodistribution. No significant focal lesion or abnormal focal FDG uptake noted.

[Note: All brain lesions may not be apparent on 18F-FDG PET/CT and MRI brain may be performed, if clinically indicated.]

HEAD AND NECK:

Non FDG avid mucosal thickening is seen in posterior wall of nasopharynx and nasal cavity, likely inflammatory.

Dental related artefacts are noted partially obscuring the loco-regional anatomy.

Mild FDG uptake is seen in bilateral palatine tonsils with few bilateral tonsilloliths and bilateral level IB & II cervical lymph nodes, likely infective / inflammatory.

Mild FDG uptake is seen in few subcentimetric right supraclavicular lymph nodes (measuring 0.9 x 0.7cm, SUVmax 2.3).

Normal physiologic FDG distribution is seen in rest of the head-neck region.

Visualized paranasal sinuses and skull base appear normal. The suprahyoid neck, infrahyoid neck, thyroid, vascular structures and thoracic inlet do not show any obvious abnormality.

THORAX:

Non FDG avid subcentimetric bilateral axillary lymph nodes are noted, likely reactive etiology.

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Mata Amritanandamayi Marg, Sector 88, Faridabad, Haryana 121002

Phone : +91 129 2851234 | Email: fbd@amritahospitals.org www.amritahospitals.org/fbd

www.amritahospitals.org/TDD

NAME	VIPAN KUMAR GARG	STUDY DATE	12.02.2024
AGE / SEX	66Years /Male	MRD NO.	112145
REFERRING DEPT	OPD, Medical Oncology	Procedure	Whole body 18F-FDG PET-CT
REPORTED ON	13.02.2024	REFERRED BY	Dr. Prashant Mehra

Increased FDG uptake is seen in multiple irregular fibronodules, few arranged in tree-in-bud pattern with adjacent ground glass haziness in the left lung upper & lower lobes (SUVmax 3.7).

Non FDG avid ground glass haziness is seen in right lung lower lobe.

Non FDG avid subpleural atelectatic changes are seen in the right middle lobe and bilateral lung lower lobes. Non FDG avid tiny nodule is noted in the right lung upper lobe, superior basal segment of right lung lower lobe. Few non-FDG avid thin walled air cysts are seen in bilateral lung parenchyma. FDG avid heterogeneously enhancing enlarged lower paratracheal, subcarinal and bilateral hilar (on the right measuring 2.0 x 1.7cm, SUVmax 5.3) lymph nodes are noted, few of the lymph nodes show internal calcifications.

Post surgery (hiatus hernia repair) status.

There is trace pericardial effusion.

Coronary artery calcifications are noted. Mild atheromatous calcifications are noted in the thoracic aorta, abdominal aorta & its branches.

Physiologic FDG uptake is seen in the myocardium. Large airways are within normal limits.

ABDOMEN-PELVIS:

Hepatomegaly is noted (liver measures 17.7cm in craniocaudal extent). Non FDG avid relative well defined hypodense lesion is seen in segment III (measuring 1.1 x 1.1cm), favours benign etiology. No obvious focal FDG avid lesion / bilobar IHBR dilatation noted. No definite contrast filling defect is seen in the portal and hepatic venous systems.

Gall bladder is partially distended without any obvious abnormality. [*Note: USG is the modality of choice for evaluation of gall stones].

Visualized stomach is poorly distended. Mild bilateral perinephric fat stranding is noted. **Prostate appears enlarged (measures 5.1 x 5.5cm, AP x TR).** No obvious focal FDG uptake noted.

Normal FDG distribution is noted in the liver, spleen, gastrointestinal tract, kidneys and urinary bladder. The spleen, kidneys, adrenals, bowel, urinary bladder and pancreas appear normal. Omentum and peritoneum show no abnormality. No ascites is noted. No structural abnormality noted in the abdominal wall. No obvious FDG avid abnormal lymphadenopathy noted in the abdomen-pelvis.

MUSCULOSKELETAL SYSTEM AND OTHERS:

Degenerative changes are seen in the spine. Sacralisation of L5 vertebra is noted. Mild FDG uptake is seen in periarticular regions of bilateral shoulder joints, likely inflammatory. Non

FDG avid benign sclerotic bone islands are seen in the head of left femur, left pubic symphysis and left acetabulum.

Increased FDG uptake is seen in pathological fractures involving: right 6th rib laterally (with soft tissue component, SUVmax 5.4) and left 8th rib posteriorly (SUVmax 2.9); mild focal FDG uptake at the right 3rd rib costo-chondral junction (SUVmax 2.0).



AMRI HOSPITAL FARIDAkot

NAME	VIPAN KUMAR GARG
AGE / SEX	66Years /Male
REFERRING DEPT	OPD, Medical Oncology
REPORTED ON	13.02.2024

Increased FDG uptake is seen in multiple irregular fibronodules, few arranged in tree-in-bud pattern with adjacent ground glass haziness in the left lung upper & lower lobes (SUVmax 3.7).

Non FDG avid ground glass haziness is seen in right lung lower lobe.



NAME	VIPAN KUMAR GARG	Embrace Health	12.02.2024
AGE / SEX	66Years /Male	MRD NO.	112145
REFERRING DEPT	OPD, Medical Oncology	Procedure	Whole body 18F-FDG PET-CECT
REPORTED ON	13.02.2024	REFERRED BY	Dr. Prashant Mehta

Increased FDG uptake is seen in collapse compression involving D6 vertebra with evidence of paravertebral soft tissue component (SUVmax 4.5).

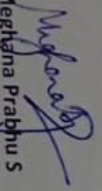
Non FDG avid lytic lesions associated with vertebral collapse involving D1 and D2 is noted.

Few other non FDG avid lytic lesions are noted in the following skeletal sites: sternum, head of right femur and multiple dorsal-lumbar-sacral vertebrae.
Physiologic FDG distribution is seen in rest of the axial and appendicular skeleton.

IMPRESSION: Case of multiple myeloma, post chemotherapy with increase in kappa FLC during follow-up. PET/CT reveals:

1. Metabolically active lytic lesions involving few skeletal sites as described (pathological fractures involving right 6th & left 8th ribs, collapsed D6 vertebra) - residual / recurrent neoplastic disease involvement.
2. Non-metabolic lytic lesions in multiple other skeletal sites as described above; D1 & D2 vertebral collapse with no significant metabolic activity.
3. Also noted:
(i) Metabolically active multiple irregular fibronodules, few arranged in tree-in-bud pattern with adjacent ground glass haziness in the left lung with multiple mediastinal & right supraclavicular lymph nodes - favors active infective/inflammatory etiology.
(ii) Hepatomegaly.
(iii) Prostatomegaly.
4. No definite scan evidence of abnormal metabolically active disease in rest of the body in this study.

**Previous FDG PET/CT scan CD not available for comparison (done at PGI, Chandigarh, dated 10.08.2023).*


Dr. Meghna Prabhuss
Assistant Professor
Department of Nuclear Medicine

Dr. Abhishek Behera
Assistant Professor
Department of Nuclear Medicine

(Please carry report and CD on your next visit for comparison)



Embrace Good Health

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MRD No: 112145

Gender: Male

Age: 66Y 10M 14D

Name: VIPAN KUMAR GARG

Visit Type: OP

Visit Code: OP0001

Service: Multiple Myeloma

RESULTREPORT

Findings

Sample ID: FCM - 67/2024

Clinical Detail: Follow up case of Multiple Myeloma

MRD Assessment time point: On maintenance Tx

Type of Sample: Bone marrow aspirate

Sample Quality: Fresh (<24 hrs) and adequate.

Methodology: Flow cytometry, Stain-Lyse-Wash (Beckman Coulter - Dx Flex)

Gating Strategy: Multicolor flow cytometry using CD45 VS side scatter gating followed by hierarchical gating.

Analysis Software: Kaluza Software (ver 2.1)

No of events acquired: 45,00,000

No of viable events (FSC Vs SSC): 96%

CD38+CD138+ Events: 0.15%

Report Summary: Flowcytometric immunophenotyping done on bone marrow aspirate shows a total of 0.15% plasma cells with immunophenotype as follows:

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2. APC (Aberrant Plasma Cells): 0.1%; Kappa restricted plasma cells displaying two distinct subpopulations:
subpopulation I: CD19+CD27+CD56+
subpopulation II: CD19-CD27-CD56+

Conclusion: Sample is positive for MRD (0.1%)

Comments

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MRD No: 112145

Name: VIPAN KUMAR GARG

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Printed On: 11/01/2024 11:11:28
Printed At: Multitest Diagnostic



Embrace Good Health

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PAN NO: AAATM2403M , Appointment call: 01292851234
Mata Amritanandamayi Marg, Sector 88, Faridabad, Haryana 121002

MRD No: 112145

Gender: Male

DOB: 30/03/1957

Name: VIPAN KUMAR GARG

Visit Type: OP

Visit Code: OP0001

BONE MARROW ASPIRATION AND BIOPSY

Ordered Date & Time: 10/02/2024 13:58

Sample ID: 0189HA240210

Sample Collected D & T:
10/02/2024 15:40

Sample Recd. D & T: 10/02/2024
15:42

Result Certified D & T: 14/02/2024
10:14

Peripheral Blood (Smear)
(Methodology - Light Microscopy)

Findings:

Peripheral smear shows normocytic, normochromic anemia with mild thrombocytopenia.

BONE MARROW ASPIRATION and IMPRINT SMEAR REPORT (BM - 35/24)

Marrow Particles: Present

Cellularity: Cellular

M:E Ratio: 1.28:1

Myelogram:

Blast	PM	MY	Mm	N+B	L	M	Eo	Baso	Plasma Cells	Eryth - roid
01	03	15	08	23	08	00	00	00	03	39

Erythropoiesis: Normoblastic erythroid reaction

Myelopoiesis: Normal sequential maturation

Megakaryopoiesis: Megakaryocytes are adequate in number and normal in morphology.

Lymphocytes: Normal in number and morphology.

Plasma cells: Mildly increased in number constituting ~3% of total cellularity, including reactive forms.

Bone Marrow Biopsy No.: AH/434/24

Gross: Received multiple bony cores measuring 0.2 cm to 1.8 cm.

Microscopic: Bone marrow biopsy is adequate for evaluation showing bony trabecular with >10 intertrabecular spaces.

Cellularity: Normocellular (60-65%)

Report

Bone marrow biopsy shows trilineage hematopoiesis with normal distribution of hematopoietic cells.

Plasma Cells: There is no appreciable increase in plasma cells OR focal clustering.

Impression:

Bone marrow appears to be in morphological remission in a follow-up case of multiple myeloma.
Correlate with MRD.

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Mata Amritanandamayi Marg Sector 88, Faridabad 121002 Haryana



MRD No: 112145

Page 1 of 2

Name: VIPAN KUMAR GARG

Printed On: 22/02/2024 13:05:05
Printed At: Medical Oncology

TATA IMA Technologies Pvt. Ltd
LABORATORY, 2nd Floor, B-228,
Gurgaon Industrial Area, Phase - 1,
New Delhi, 110020
www.ima.com/ima
info@ima.com
CN: U714002019PTC378228

REGISTERED OFFICE:
LEVEL 3, Vasant Square Mall,
Pocesse V, Sector B,
Vasant Kunj, New Delhi - 110070

नेहरू चिकित्सालय NEHRU HOSPITAL
स्नातकोत्तर चिकित्सा शिक्षा एवं अनुसंधान संस्थान, चण्डीगढ़
POSTGRADUATE INSTITUTE OF MEDICAL EDUCATION AND RESEARCH
CHANDIGARH

बाहरी रोगी कार्ड OUT PATIENT CARD

V. K. Garg

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Age 19539.

सांक्षिपिक विवरण / सांक्षिपिक विवरण PROVISIONAL DIAGNOSIS	
तारीख व हस्ताक्षर DATE & INITIALS	

आदेश प्राप्त होने व उपचार
TREATMENT AND INVESTIGATIONS ORDERED
SUPPLY FOLLOWING DRUGS OR
EQUIVALENT GENERIC DRUGS

19/07/22.

1 PEB

1 kg liquid chain
away

Mkt. 21 quin

low pressure

excess 2
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गोपनीयता का प्रतीक 21 दिन के अंदर करवाया जाएगा है।
यदि गोपनीयता का प्रतीक 21 दिन के अंदर नहीं किया गया है तो गोपनीयता का प्रतीक 21 दिन के अंदर नहीं किया गया है।

2018-19-2019-20

Dr

9454105L88 4451M 2011.2.11

May 2023

REGISTERED OFFICE:
LEVEL 3, Varant Square Mall,
Pocher V, Sector B,
Varant Kunj New Delhi - 110070

Technologies Pvt. Ltd
2nd Floor, B-22b,
Pocher V, Sector B,
Varant Kunj New Delhi - 110070

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Gurg 166 Y/M

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K Gurg 166 Y/M

LE NO:

CR No. 20140513
V K Gurg 166 Y

SAMPLE NO:

CR No. 2014
V K Gurg

SAMPLE NO

CR No
V K

SAMPLE

रोगी का रोगी का
DATE & INITIALS

TREATMENT AND INVESTIGATION ORDERED

SUPPLY FOLLOWING DRUGS OR
EQUIVALENT GENERIC DRUGS

19/12/22

MAD -ve
EM -ve

Low Dose Ved.
regiment

Monthly review

CC / BP / Calcium

केवल जाँच के लिए / FOR INVESTIGATIONS ONLY

ऑनलाइन लैब रिपोर्ट वेबसाइट pgimer.edu.in/ors.gov.in पर उपलब्ध है
Lab test reports are available on website pgimer.edu.in/ ors.gov.in

Adm but

for auto
Tx
mend 1/
cmm

21/12/22

Maintenance

Zoledronic
acid
11mg once

1mg OD x 11 day
- Cap ben aldonic
murex etc.
- Can Deni or /ata /urb / Etacipin D

Ht = 175 cm

wt = 74 kg

TATA IMG Technologies Pvt. Ltd
LABORATORY: 2nd Floor, B-225,
Okhla Industrial Area, Phase - 1,
New Delhi, 110020

REGISTERED OFFICE:
LEVEL 3, Vasant Square Mall,
Pocket V, Sector B,
Vasant Kunj New Delhi - 110070

तारीख व हस्ताक्षर
DATE & INITIALS

बताई गई जांचे व उपचार
TREATMENT AND INVESTIGATION ORDERED

RECORD DEPARTMENT

SUPPLY FOLLOWING DRUGS OR
EQUIVALENT GENERIC DRUGS

AUG 2023

21/8/22

Rm MRO -ve

DETECT - non EDC
and

3 mar

Iy Calizemil
20mg qd
twice monthly
Tat Acivir DT
4mg bd.

केवल जाँच के लिए / FOR INVESTIGATION ONLY
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Lab test reports are available on website pgimer.edu.in / ors.gov.in

Calcium / vit D

Iy Zoledroni
and vms
iv 1 Pmt

Adv to
carry all,
mediums.
with him
gone to
USA

ll in m hls
21/1/24

तारीख व हस्ताक्षर
DATE & INITIALS

बताई गई जांचे व उपचार
TREATMENT AND INVESTIGATION ORDERED

SUPPLY FOLLOWING DRUGS OR
EQUIVALENT GENERIC DRUGS

By Zoledroni acid
4mg IV
1 time
monthly
x 2 more
then once in
3 months

Calcium / vit-D
supplemental

Cbc / RFT / Calcu,
LFT / RFT / 1 hr urine
Chem assay
in May 23

Admit
for renal
dialysis

तारीख व हस्ताक्षर
DATE & INITIALS

बताई गई जांचे व उपचार
TREATMENT AND INVESTIGATION ORDERED

SUPPLY FOLLOWING DRUGS OR
EQUIVALENT GENERIC DRUGS

30/11/23
doing well
Planning dental extraction
Clap Zoledroni
x 12 ml

Cav. benalidensin

Batizemil
doing 4c
twice monthly

Cav. Desir
4mg 6d

Calcium / vit-D
supplemental

RFT
Cbc / RFT / Calcu
myeloma test up
Aug 23

PET-CT
RFT
MRD

C/o DR. Mohan Sahwa USA

नेहरू चिकित्सालय NEHRU HOSPITAL
स्नातकोत्तर चिकित्सा शिक्षा एवम् अनुसंधान संस्थान, चण्डीगढ़
POSTGRADUATE INSTITUTE OF MEDICAL EDUCATION AND RESEARCH
CHANDIGARH

बाहरी रोगी कार्ड OUT PATIENT CARD

SENIOR CITIZEN

CR No. 58 0112 19810462Y
Date 30-03-2023
Name No. 201405736412
Age / Sex : 65 Y M
Father Name : Sri Bhushan Gure Category : General
Address : 323 SEC 9 P.K.L.
Rafvans India
Department : Clinical Hematology & Medical Oncology
Unit / Clinic : Hematology Clinic/PM.AK.GP.DL.AJ
Room No : 3024
File No : CHMAHC19539 Serial No : 76
Unit Date : Mon.Tue.Thu Amount (Rs) : 10/-
Renewal

सामयिक निदान/तात्कालिक निदान
PROVISIONAL DIAGNOSIS

तारीख व हस्ताक्षर
DATE & INITIALS

बताई गई जांचे व उपचार
TREATMENT AND INVESTIGATIONS ORDERED

SUPPLY FOLLOWING DRUGS OR
EQUIVALENT GENERIC DRUGS

30/3/23

Maintenence
- treatment

(here Intolerant)

In Butazolidin
20mg q/c
twice monthly

Tal Aciriv DT
400mg b.i.

जन्म तथा मृत्यु का पंजीकरण 21 दिन के अन्दर करवाना अनिवार्य है।
पी.जी.आई. में हुए जन्म तथा मृत्यु के प्रमाण पत्र सी.आर.डी. विभाग में उपलब्ध हैं